

CUHK Pharmacy Teaching Cases

Apricot AI's interpretation of the four case packs and the points requiring faculty confirmation before implementation

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Purpose of this brief

This document records our current reading of the four teaching cases so that role behaviour, clinical content, assessment logic and feedback reporting can be configured correctly. It is not intended to replace CUHK's approved teaching references. Items marked for confirmation should be resolved before the relevant case is treated as faculty-approved content.

Executive summary

The educational intent of all four cases is clear. Cases 1 and 2 are drug-information telephone stations in which the simulated role is a resident doctor and the student is a junior pharmacist. Cases 3 and 4 are patient-counselling stations. Case 3 is a follow-up discussion with PKC about PRN furosemide and statin therapy; Case 4 is a high-risk methotrexate counselling discussion that, based on the task wording and all demonstration videos, is conducted with JKC's daughter or caregiver.

The source pack is sufficient to model the interaction flow for all four cases. It is not yet sufficient to freeze every scoring rule. Case 3 does not contain a complete matching rubric or the new prescription. Case 4 contains a role inconsistency, and its rubric is located in the Case 3 folder. The latest faculty email resolves several Case 3 and Case 4 video interpretations, but it provides calibration observations rather than numeric weights, grade boundaries or a complete rubric.

Recommended implementation status

Cases 1 and 2 can proceed to structured case build once the listed content points are confirmed. Case 3 can incorporate the newly confirmed allergy, grapefruit, oedema-assessment and rhabdomyolysis rules, but should remain in draft until CUHK supplies the final prescription and rubric. Case 4 can incorporate the confirmed separate-day folic-acid and blood-monitoring rules, but its role, prescription intent, renal/interacting-medicine expectations, missed-dose answer and scoring consequences must be confirmed before scoring is released.

1. Review basis and case portfolio

Our review covered the complete material supplied for the four cases: the scenario and assessment documents, all supporting clinical references and exhibits, and all demonstration recordings. For the eight Case 3 and Case 4 segments specified in CUHK's latest email, the faculty rating and observations are treated as the current controlling calibration. Other video labels remain source metadata rather than proof that every statement in a recording is clinically or pedagogically correct. The calibration comments do not by themselves define a numeric score.

SOURCE FILES	DEMONSTRATION VIDEOS	WORD DOCUMENTS	PDFS / PAGES
42	18	9	15 / 524

Portfolio at a glance

Case	Simulated role	Primary capability	Current readiness
1. Fungal Toe	Resident doctor	Drug-information process; contraindication; interaction; dose and evidence synthesis	Core structure is usable; diagnosis and renal-function wording need confirmation
2. Migraine	Resident doctor	Acute treatment selection in complex cardiovascular and renal disease	Core structure is usable; local availability and selected answer wording need confirmation
3. Diuretics	Patient (PKC)	PRN diuretic use; heart-failure self-management; statin counselling	Several safety points are confirmed; prescription, complete rubric and numeric rules remain missing
4. MTX	Caregiver / daughter	Once-weekly methotrexate safety; folic acid; monitoring; teach-back	Separate-day folic acid and blood monitoring are confirmed; role, missed-dose answer and scoring rules remain open

How we have interpreted conflicting evidence

The written scenario defines the patient and task context unless a later faculty-approved version supersedes it.

The written rubric defines intended assessment points, but a clinically unsafe or internally inconsistent rule is flagged rather than silently implemented.

Demonstration videos inform realistic role behaviour, prompt timing and common student errors; they are not used as verbatim gold-standard scripts.

For high-risk drug names, doses, frequencies and units, the source document or on-screen prescription takes precedence over uncorrected speech recognition output.

Cross-case implementation reading

The two drug-information cases require a requester-doctor state model: the doctor provides private facts only when appropriately asked, challenges an unsuitable recommendation and accepts a call-back after the student has searched the evidence. The two counselling cases require a patient or caregiver state model that can express practical concerns, test whether instructions are understandable and complete a genuine teach-back. The platform should therefore store the student role and simulated role explicitly for each case rather than infer them from generic “doctor” and “patient” labels.

2. Case 1 — Drug Information: Fungal Toe

This is not primarily an itraconazole dose-retrieval exercise. It tests whether the student recognises that the requested drug is unsuitable for this particular patient, gathers enough context to reframe the question and returns a defensible alternative using more than one information source.

Case fact	Current interpretation
Patient	PKC, 78-year-old woman; admitted to PWH Ward 10A, Bed 5; NKDA.
Relevant history	HFrEF with LVEF 35%, hypertension, type 2 diabetes, hyperlipidaemia and impaired renal function during admission.
Key medicines	Atorvastatin 40 mg daily, Entresto, spironolactone, dapagliflozin and other admission medicines.
Nail presentation	Approximately eight of ten toenails affected for several months; great-toe pain while walking; no previous oral or topical treatment.
Student / role	Junior pharmacist / resident doctor Dr Anson Lo, simulated telephone consultation in English or Cantonese.
Time pressure	The patient is expected to be discharged; the requester wants an answer as soon as possible.

Our understanding of the intended station

The student identifies the requester, urgency and contact route, then clarifies whether the real question is simply a dose check or the safest treatment for this patient.

The student elicits disease extent, duration, pain, prior treatment and practical feasibility. The doctor should not volunteer the full history at the outset.

The student recognises the heart-failure concern with itraconazole and the interaction with atorvastatin 40 mg, searches at least two appropriate sources and calls back with a patient-specific recommendation.

After the main question is resolved, the doctor asks about carbamide for thickened nails. The intended answer is that carbamide is urea, acts as a keratolytic and is available in a 40% preparation for this use.

Current expected clinical conclusion

Itraconazole should not be treated as a routine dose-selection answer in this patient because of HFrEF / ventricular dysfunction.

The itraconazole–atorvastatin interaction must be identified. The supplied material limits atorvastatin to 20 mg daily if co-administered and prompts consideration of an alternative statin or an alternative antifungal.

The current rubric expects oral terbinafine 125 mg once daily for 12 weeks in the stated renal-function range. This is the expected case answer, subject to faculty confirmation of how the acute rise in creatinine and the need for diagnostic confirmation should be handled.

Topical treatments are deliberately unattractive in the case because eight toenails are involved and prolonged application would be burdensome. The role should challenge the practicality of a topical-only proposal.

Assessment logic we would implement

The existing rubric supports a safety-first assessment. The following items act before ordinary communication or process marks:

Failure, despite prompting, to identify the heart-failure concern with itraconazole.

Failure, despite prompting, to identify the itraconazole–statin interaction.

An incorrect dose or treatment duration, an incomplete answer due to time management, or unprofessional conduct.

The present grade caps are C+ if either major safety point is identified only after prompting, B– if terbinafine is not proposed, and a one-subgrade deduction if the requester and contact details are not identified. Correctly explaining both carbamide points earns the stated bonus. We would preserve these as structured rules rather than infer a grade from transcript similarity.

What the demonstration videos add

The recordings show that a student may eventually state the expected dose after first giving a wrong dose, or may name terbinafine without explaining why itraconazole is unsuitable. The score therefore needs to retain the sequence of the answer: initial error, prompt, correction and final executable recommendation. The “Best” recording should be used as a flow reference, not as a word-for-word gold transcript, because its explicit coverage of the heart-failure and statin issues is not complete enough to define every required criterion.

Case 1 position before faculty confirmation

The station objective, role behaviour, main safety gates and existing grade caps are sufficiently clear to build. Diagnostic confirmation, renal-function wording and a few editorial inconsistencies should be resolved before the case is labelled final.

3. Case 2 — Drug Information: Migraine

The apparent request is to check “Cafergot two tablets when required”. The intended task is broader: the student must reject ergotamine for a patient with recent myocardial infarction and significant cardiovascular disease, avoid other vasoconstrictive choices and recommend a feasible acute treatment that also fits the patient's renal function and local context.

Case fact	Current interpretation
Patient	PKC, 72-year-old Chinese man; NKDA; approximately one migraine attack per week.
Relevant history	HFrEF with LVEF 30%, STEMI in December 2024, hypertension, CKD, diabetes, depression/anxiety and migraine.
Renal / laboratory context	Serum creatinine 154 micromol/L; estimated creatinine clearance approximately 39 mL/min; sodium 127 mmol/L.
Previous acute treatment	Paracetamol and naproxen have recently been ineffective.
Student / role	Junior pharmacist / resident doctor Dr Anson Lo, simulated telephone consultation.
Follow-up question	The doctor asks for the usual IV magnesium sulfate dose for a severe acute migraine.

Our understanding of the intended station

The student establishes that Cafergot contains ergotamine and caffeine and that the request concerns acute, as-needed treatment rather than prevention.

The student asks about attack frequency, current symptoms, previous treatment and allergies, then prioritises the patient's recent STEMI, coronary disease, HFrEF, hypertension and renal impairment over a narrow dose check.

If the student focuses only on citalopram or metoprolol interactions, the doctor should prompt for the more important patient-level contraindication. If an NSAID or triptan is proposed, the doctor should challenge the previous failure or cardiovascular safety.

The present rubric expects rimegepant 75 mg as a single as-needed dose, maximum 75 mg in 24 hours, with no dose adjustment at creatinine clearance of at least 15 mL/min.

Current expected clinical conclusion

Cafergot should be rejected because ergotamine is unsuitable in the context of recent STEMI / coronary disease, heart failure and hypertension; renal impairment adds to the concern.

The citalopram and metoprolol interactions are relevant but secondary. Identifying an interaction alone does not resolve the case.

Triptans are not an acceptable substitute in this case because of the recent myocardial infarction and coronary disease. NSAIDs have already failed and present additional cardiovascular / renal concerns.

Rimegepant is the case pack's expected oral acute option. We would describe it as the preferred answer under the case assumptions, not as the universally “best and only” option.

Assessment logic we would implement

The existing auto-fail criteria are failure, despite prompting, to identify the ergotamine contraindication; failure to provide a suitable alternative after prompting; materially incorrect information; failure to complete the answer; or unprofessional conduct. The current grade caps are C+ when the heart-failure / Cafergot issue is identified only after prompting, B- when the CGRP option is identified only after prompting, and B when the magnesium follow-up is not answered. Requester/contact identification carries the stated one-subgrade deduction.

For automated scoring, an unsafe triptan or NSAID proposal should remain visible even if the student later withdraws it. The report should distinguish an unprompted safe alternative, a prompted correction and an unresolved recommendation.

What the demonstration videos add

The videos provide useful examples of common failure patterns: concentrating on serotonin-related interactions, selecting a triptan before considering the recent MI, and spending time searching without returning an actionable answer. They also reveal a conflict in the magnesium follow-up: the “Best” recording gives a different dose/rate from the written rubric. In addition, the rubric's route descriptions for several CGRP monoclonal antibodies should not be copied into the final case without correction.

Case 2 position before faculty confirmation

The station flow and major safety gates are sufficiently clear to build. CUHK should confirm the accepted acute-treatment set, the local-availability convention and the authoritative magnesium answer before the case is locked for assessment.

4. Case 3 — Patient Counselling: PRN Diuretic and Statin

This is a Cantonese counselling station with PKC one month after the earlier heart-failure encounter. The student must convert a PRN furosemide instruction into practical self-management advice, explain when heart-failure symptoms require medical attention and provide safe, understandable counselling for newly initiated statin therapy.

Case fact	Current interpretation
Patient	PKC, 72-year-old man with HFrEF, recent STEMI, CKD, diabetes, depression/anxiety and migraine.
Current status	Previously had two episodes of leg swelling; currently described as euvolaemic after reducing salt and fluid intake.

Case fact	Current interpretation
New therapy	Furosemide / frusemide 20 mg once daily when required for leg oedema, plus planned statin initiation.
Student / role	Junior pharmacist / PKC as the simulated patient; expected language is Cantonese.
Material gap	No matching new prescription is included. The exact statin and dose cannot be frozen from the videos alone.
Rubric gap	The file labelled as the Case 3 scenario begins with heart-failure/statin objectives but then contains the Case 4 methotrexate rubric.

Our understanding of the intended station

The simulated patient should begin with a natural clinic handover — for example, that the nurse asked him to speak to the pharmacist before leaving — and should reveal concerns in response to the student's counselling. He should not deliver a checklist of questions at the outset. The role may ask what “when required” means, how to recognise oedema, what symptoms suggest worsening heart failure, how much salt or fluid is appropriate, why a statin is needed when he feels well, and what to do after a missed dose.

Candidate counselling content from the current pack

PRN furosemide and heart-failure self-management

Confirm identity, allergies, the medicine and the intended PRN trigger. The latest faculty calibration confirms that failure to ask about drug allergy at the beginning is an auto-fail in this station. The prescription context is leg oedema; it should not be described as an unconditional daily dose.

Explain morning or daytime administration, increased urination and practical fall-risk precautions.

Discuss dehydration, dizziness or hypotension, marked weakness, palpitations and possible electrolyte disturbance in language the patient can understand.

Escalate persistent oedema, worsening breathlessness, respiratory distress or other signs of decompensation rather than encouraging repeated self-escalation of the diuretic.

Give objective ways to recognise fluid retention: daily weighing and checking for pitting at the ankle, rather than relying only on a subjective feeling of heaviness or swelling. The action threshold still requires faculty confirmation.

Reinforce salt reduction, daily monitoring and activity in an individualised manner. A fixed fluid limit should not be invented when no such instruction is supplied.

Statin counselling

Explain LDL reduction and secondary prevention of myocardial infarction and stroke, including the need for daily adherence despite the absence of immediate symptoms.

Discuss unexplained severe muscle pain or weakness, dark urine, jaundice and other signs that warrant prompt clinical review. Rhabdomyolysis is rare but serious and must not be dismissed without explaining the warning signs and required action.

Give a clear missed-dose instruction: take it when remembered if the next dose is not close; otherwise skip it and do not double the dose.

Do not teach a grapefruit restriction for rosuvastatin. The latest faculty calibration explicitly identifies this advice as incorrect for the medicine used in the station.

Complete teach-back by asking PKC to explain when he would take the diuretic, what would prompt urgent help and how he will take the statin.

What the demonstration videos add

Faculty rating	Specified cut	Calibration observations
Good	00:00:30–00:08:50	Faculty-rated Straight A performance covering approximately 90% of the expected points. This is a qualitative high-performance anchor, not a numeric score or a complete rubric.
Below average — auto-fail	01:01:28–01:09:02	Automatic fail because drug allergy was not asked at the beginning. The student needed prompting for heart-failure symptoms and missed-dose advice, and incorrectly stated that grapefruit cannot be taken with rosuvastatin.
Below average	00:22:36–00:30:39	Did not verify the patient's name and HKID; needed prompting for lifestyle advice, especially fluid restriction; gave the incorrect grapefruit advice; and needed prompting for the missed dose.
Fail	00:48:26–00:56:48	Choppy and disorganised; did not verify name/HKID or ask about allergy; minimised rhabdomyolysis without explaining its serious warning signs; gave incorrect grapefruit advice; and offered only subjective swelling language rather than weighing or checking for pitting oedema.

Confirmed implications of the latest faculty calibration

Failure to ask about drug allergy at the beginning is an auto-fail for this station.

Rosuvastatin should not be presented as interacting with grapefruit in the manner stated in the lower-performing recordings.

Objective fluid-status advice should include daily weighing and/or checking for pitting ankle oedema; the precise action threshold remains open.

Rhabdomyolysis must be framed as rare but serious, with symptoms and escalation advice. Simply calling it rare is insufficient.

The faculty comments confirm that prompting matters, but they do not state the numerical deduction or grade cap for each prompt. Omission of name/HKID is important, but the email does not establish that identity omission alone is an auto-fail.

Case 3 position before faculty confirmation

We can draft the patient behaviour and implement the faculty-confirmed allergy, grapefruit, oedema-assessment and rhabdomyolysis rules. We should not publish a CUHK numeric score until the prescription, complete matching rubric, remaining safety rules, weights, prompt caps and grade boundaries are supplied or approved by faculty.

5. Case 4 — Patient Counselling: Methotrexate and Folic Acid

This is a high-risk, once-weekly medicine counselling station. The central outcome is not the number of adverse effects recited; it is whether the caregiver can safely describe the methotrexate day, the folic-acid day, the missed-dose plan, the warning symptoms and the monitoring arrangement in her own words.

Case fact	Current interpretation
Patient	JKC, 82-year-old man; HKID A123456; NKDA; lives with his daughter.
Relevant history	Rheumatoid arthritis, CKD, atrial fibrillation, previous stroke presentation, hypertension, diabetes and dyslipidaemia.
Renal context	Serum creatinine approximately 158–165 micromol/L; estimated creatinine clearance approximately 29 mL/min.
Relevant existing medicines	Ibuprofen 400 mg every six hours when required and pantoprazole 40 mg daily are listed in the pre-admission history.
New prescription	Methotrexate 2.5 mg once weekly for four weeks and folic acid 5 mg once weekly for four weeks.
Student / role	Junior pharmacist / daughter or caregiver collecting medicine for her father, according to the task wording and all four videos.

Our understanding of the intended station

The caregiver should initially know that the medicines are for her father but should not already understand the regimen. She should ask practical questions about why two medicines are needed, whether they are taken on the same day, what happens if the weekly dose is forgotten, whether blood tests are required and when an adverse effect is serious. The role should be able to complete teach-back and should reveal relevant OTC or prescription medicine use only when the student asks appropriately.

Current expected counselling content

Introduce the pharmacist's role; confirm the patient's identity, allergy status, current condition and full medicine list, including OTC products, Chinese medicines and supplements.

Explain that methotrexate is a disease-modifying antirheumatic drug used to control rheumatoid arthritis and slow disease progression. Emphasise once-weekly use, never daily use.

Explain folic acid as a measure to reduce methotrexate toxicity. The latest faculty calibration confirms that it should be taken on a different day from methotrexate; same-day advice is incorrect. It must not be described as being 'for the brain'.

Discuss infection / bone-marrow warning signs, unusual bruising or bleeding, mouth ulcers, gastrointestinal or liver-related symptoms, dark urine, breathlessness and other respiratory symptoms requiring review.

Explain that blood monitoring is required. The current rubric lists FBC, renal and liver function every two to three months and also lists pulmonary function testing; the exact tests, frequency and role of pulmonary testing still require clarification.

Explain that benefit may take approximately three to six weeks, give the faculty-approved missed-dose rule and explicitly prohibit double or closely repeated dosing.

Address alcohol, OTC analgesics, vaccination and infection precautions to the extent intended by the station, then complete a genuine caregiver teach-back.

Assessment logic in the recovered rubric

The matching methotrexate rubric is found in the Case 3 “Diuretics scenario” file rather than in the Case 4 folder. It uses a Yes/No checklist and identifies the following auto-fail conditions: failure to ask about allergy; inaccurate or potentially harmful advice; unprofessional behaviour; failure to emphasise weekly methotrexate; or failure to cover key counselling because of time management. It does not provide numeric weights, a total score, grade bands or prompt-dependent caps.

What the demonstration videos add

Faculty rating	Specified cut	Calibration observations
Good	00:35:33–00:43:44	Pace was appropriate and most points were covered. The student could not answer how to handle a missed dose but was otherwise acceptable. This shows that failure to answer the missed-dose question is important but is not, by itself, an automatic fail.
Average	00:06:30–00:14:45	Overall acceptable, but unable to answer the missed-dose question. Blood monitoring required prompting, after which the student incorrectly stated that no blood draw was needed.
Below average	01:36:53–01:44:54	Initially disorganised: asked about allergy before identifying the patient; could not answer the missed-dose question; needed prompting for blood monitoring; and incorrectly insisted that folic acid was 'for the brain' even after prompting.
Poor (fail)	02:57:36–03:07:00	Did not verify the patient's name or HKID; gave an incorrect methotrexate mechanism involving reduction of folic acid; could not answer the missed-dose question; and incorrectly advised folic acid on the same day as methotrexate rather than on a separate day.

Confirmed implications of the latest faculty calibration

Methotrexate and folic acid should be taken on separate days; same-day counselling is incorrect.

Blood monitoring is required; stating that no blood draw is needed is incorrect. The exact tests and frequency remain open.

Explaining folic acid as being 'for the brain' is incorrect, especially when the student persists after prompting.

The missed-dose question is a repeated differentiator across all four clips. However, because the Good performance still could not answer it, inability alone must not be implemented as an automatic fail without a further faculty decision.

The email identifies sequence and identity-verification problems, but it does not define the isolated deduction or grade cap for asking about allergy before identity verification or for omitting name/HKID.

Clinical intent that must be settled

The supplied prescription is methotrexate 2.5 mg once weekly. Current product information commonly gives a higher adult rheumatoid-arthritis starting dose and warns that renal impairment increases exposure and toxicity. JKC's estimated creatinine clearance is approximately 29 mL/min, and the case history includes ibuprofen and pantoprazole. Faculty should confirm whether the student is expected only to counsel on a deliberately chosen low dose, or is also expected to identify a prescribing / monitoring issue before dispensing. This decision materially changes the station objective and the safety rubric.

Case 4 position before faculty confirmation

The interaction, once-weekly safety objective, separate-day folic-acid rule and need for blood monitoring are clear. The final case cannot be scored reliably until CUHK confirms the simulated identity, prescription intent, renal and interacting-medicine expectations, exact missed-dose answer, monitoring details and numeric assessment model.

6. Proposed assessment and reporting model

Across the four stations, a keyword count would not be defensible. The videos contain examples in which a student says many correct things but also gives one potentially harmful instruction. We therefore propose a layered model that mirrors the way faculty assess safety, reasoning and communication.

Layer	What is assessed	How it appears in the report
1. Safety gate	Contraindications, weekly-versus-daily use, dangerous missed-dose advice, materially wrong dose or duration, and faculty-defined allergy requirements	Pass / fail with the exact supporting utterance and timestamp
2. Clinical content	Required, optional and bonus items; accepted alternatives; correct rationale; rejected or unsafe alternatives	Item score with evidence and the faculty-approved source version
3. Prompt dependence	Whether the student identified the point independently, after a neutral prompt or only after a leading prompt	Prompt count and any resulting grade cap

Layer	What is assessed	How it appears in the report
4. Information process	Requester identification, clarification, patient-specific information collection, source use and call-back for the DI stations	Process score and missing steps
5. Communication	Clarity, professionalism, prioritisation, accessible explanations and response to questions	Observed strengths and specific improvement points
6. Teach-back	Whether the patient or caregiver can accurately restate the critical instructions	Confirmed / incomplete / incorrect understanding

Recommended faculty-facing report output

Overall score and grade only where CUHK has approved the weights and boundaries.

A separate safety outcome that cannot be cancelled by communication points.

Criterion-by-criterion evidence from the student's own words, with timestamps for voice sessions.

A distinction between an initial unsafe answer, a prompted correction and an independently safe final answer.

Concise strengths, priority improvements and a recommended practice focus.

A review flag when speech recognition is uncertain for a high-risk token such as a drug name, dose, decimal, unit, “daily” or “weekly”.

Speech handling for assessment

The current working speech configuration is Qwen ASR with a pharmacy terminology list and Azure HiuMaan for Cantonese speech output. For assessment, the platform should retain the recording, raw transcript and normalised transcript. Low-confidence drug names, doses and frequencies should be reviewed against the audio before triggering an auto-fail. This is particularly important in Cases 1 and 4, where a single number or the distinction between daily and weekly changes the safety outcome.

7. Items requiring CUHK confirmation

The latest faculty email already resolves several clinical and calibration points. Those confirmed points are recorded in Sections 4 and 5 and are not repeated below as open questions. The remaining register is intended to be answered briefly and converted directly into case configuration, rubric rules and acceptance tests.

No longer open after the latest faculty calibration

Diuretics: allergy omission at the beginning is an auto-fail; the stated rosuvastatin–grapefruit restriction is wrong; objective oedema measures and serious rhabdomyolysis counselling are expected.

Methotrexate: folic acid is taken on a separate day; blood monitoring is required; and the 'for the brain' explanation is wrong. Exact scores, prompt caps, missed-dose wording and monitoring details remain open.

ID	Area	Confirmation required
G1 	All cases	Which document is authoritative when a scenario, rubric and demonstration video conflict?
G2	All cases	Please confirm that the latest faculty email is the controlling calibration for its eight specified video segments and supersedes inconsistent earlier labels or interpretations.
G3 	All cases	Please confirm the required language for each station, the acceptable extent of Cantonese–English code-switching, and whether accent or fluency is scored separately from clinical communication.
G4 	All cases	Please confirm the prompting policy: permitted prompt sequence, number of prompts, and the grade cap after each prompt level.
G5 	All cases	Should every auto-fail override the total numeric score, and must a human reviewer approve auto-fails triggered from voice sessions?
G6 	All cases	Please confirm the required score-report fields and whether faculty require a numeric score, letter grade, safety result, evidence excerpts and recommended remediation.
G7 	All cases	Which clinical references, editions and cut-off dates should be locked as the approved case sources?
G8	All cases	May the simulated role volunteer urgent safety information, or should private case facts be disclosed only when the student asks an appropriate question?
G9 	All cases	Please confirm the official counselling-case numbering. The latest email calls MTX 'Case 1' and Diuretics 'Case 2', whereas the supplied folders label Diuretics as Patient Counselling Case 1 and MTX as Case 2.
1.1	Fungal Toe 	Please confirm that PKC is female and that all male pronouns in the checklist should be corrected.
1.2	Fungal Toe 	Should “dysphoric nail” be corrected to “dystrophic nail” throughout the case?
1.3	Fungal Toe 	Must the student recommend mycological confirmation before systemic antifungal treatment, and is this a scored item?
1.4	Fungal T 	Please confirm terbinafine 125 mg once daily for 12 weeks as the intended answer despite the recent creatinine rise and estimated CrCl of approximately 27 mL/min.
1.5	Fungal Toe 	Should the role accept another clinically defensible alternative, or is terbinafine required for a full pass?
1.6	Fungal Toe 	Please confirm the expected carbamide counselling beyond “urea / keratolytic / 40%”, including application frequency and duration if these are scored.
1.7	Fungal Toe 	May the nail photographs be used only as an exhibit, or should they be treated as diagnostic evidence within the station?
2.1	Migraine 	Should rimegepant be described as the preferred answer under the case assumptions rather than the “best and only” option?
2.2	Migraine 	Which acute alternatives, if any, should receive full or partial credit when rimegepant is not proposed?
2.3	Migraine 	What source and effective date should define Hong Kong availability: registration, hospital formulary listing or confirmed stock / procurement access?
2.4	Migraine 	Please provide the authoritative dose and infusion rate for the IV magnesium sulfate follow-up question; the rubric and “Best” video differ.

ID	Area	Confirmation required
2.5	Migraine	Should the route descriptions for erenumab, fremanezumab and galcanezumab be corrected, while retaining the teach-back point that these are not the intended acute oral PRN option?
2.6	Migraine	Is allergy confirmation mandatory whenever the student recommends an alternative medicine, and what is the consequence if omitted?
2.7	Migraine	Please confirm the exact wording to be accepted for the ergotamine safety issue: recent MI / coronary disease, HFrEF, hypertension, renal impairment, or a defined combination.
3.1	Diuretics	Please provide the final prescription used in this station, including the exact statin name, strength, dose, timing and intended duration.
3.2	Diuretics	Please provide or approve the complete matching rubric, numeric weights, grade boundaries, auto-fail rules and prompt-dependent grade caps.
3.3	Diuretics	Please define the approved PRN trigger for furosemide, the required objective oedema checks, the daily-weight action threshold and whether self-adjustment of dose is permitted.
3.4	Diuretics	What salt and fluid advice should be accepted, and is there an individual fluid-restriction target for this patient?
3.5	Diuretics	What deduction or grade cap applies when heart-failure symptoms, fluid restriction or missed-dose advice is produced only after prompting?
3.6	Diuretics	Which heart-failure red flags and urgency levels must be covered for a full pass?
3.7	Diuretics	Beyond the now-confirmed relevance of objective monitoring, are vaccination, exercise and leg elevation mandatory, optional or outside the assessed scope?
3.8	Diuretics	What scoring consequence applies if the student does not verify the patient's name and HKID? Is complete identity omission an automatic fail or a deduction / grade cap?
3.9	Diuretics	Must the student complete an actual patient teach-back, and which instructions must PKC restate correctly?
4.1	Methotrexate	Should the simulated role be JKC's daughter / caregiver rather than JKC himself? If so, please provide the preferred name, relationship and information she is expected to know.
4.2	Methotrexate	Is methotrexate 2.5 mg once weekly an intentionally selected regimen, or a prescribing issue the student is expected to identify before counselling?
4.3	Methotrexate	How should JKC's estimated CrCl of approximately 29 mL/min affect the expected student response, dispensing decision and safety rubric?
4.4	Methotrexate	Must the student identify or counsel on the listed ibuprofen and pantoprazole use, and what action is expected?
4.5	Methotrexate	Please provide the exact faculty-approved missed-dose rule, including the cut-off after two, three or four days, management near the next dose, and the different consequences of no answer versus unsafe advice.
4.6	Methotrexate	What deduction, grade cap or auto-fail applies when a student incorrectly recommends same-day methotrexate and folic acid?
4.7	Methotrexate	Please confirm whether methotrexate may be taken with or without food and whether any timing preference should be scored.
4.8	Methotrexate	Please define the required blood tests, monitoring frequency and action. Is pulmonary function testing routine, baseline-only or symptom-triggered in this case?

ID	Area	Confirmation required
4.9	Methotrexate 	Please confirm the assessed scope for alcohol, vaccination, infection precautions and OTC / Chinese medicine checks.
4.10	Methotrexate	Please provide numeric weights, grade boundaries and prompt caps, or confirm that the output should remain a checklist plus safety result.
4.11	Methotrexate 	Is prescription appropriateness assessment part of the station, or should the student assume the prescription has already been clinically validated and focus only on counselling?
4.12	Methotrexate 	What scoring consequence applies to complete omission of name/HKID, asking about allergy before patient identification, an incorrect folic-acid purpose, or persisting with that error after prompting?

8. Decisions requested to proceed

For an efficient review, we suggest that CUHK first resolves the items below. They are the decisions most likely to change case behaviour, scoring or patient-safety handling.

Confirm the official counselling-case numbering and the authority of the latest faculty calibration comments.

Approve the role mapping: doctor for Cases 1–2, PKC for Case 3, and JKC's daughter / caregiver for Case 4.

Supply the Case 3 prescription and final matching rubric.

Confirm whether Case 4 tests counselling only or includes identification of a potentially unsuitable methotrexate prescription in severe renal impairment.

Provide the exact methotrexate missed-dose answer, blood-monitoring details and scoring consequences for the calibrated errors.

Approve the Case 2 acute-treatment answer set, local-availability convention and magnesium sulfate answer.

Approve all safety gates, prompt rules, score weights and grade boundaries before numeric grading is enabled.

Nominate the authoritative reference version for every dose, contraindication, interaction and monitoring rule used in acceptance testing.

What Apricot AI will produce after confirmation

A structured case definition for each station, including visible scenario facts, role-private facts, role behaviour and prompt rules.

A faculty-approved rubric with safety gates, weighted criteria, accepted alternatives, prohibited recommendations and prompt-dependent caps.

A scoring report specification with traceable evidence from the student's conversation.

Voice and text acceptance tests covering Cantonese–English code-switching, pharmacy terminology, drug names, doses and weekly-versus-daily safety tokens.

A content-version record so any later change to a source, formulary position or faculty decision is auditable.

Proposed review outcome

Once CUHK has confirmed the register in Section 7, Apricot AI can convert the agreed content into implementation-ready case and rubric specifications without relying on assumptions that are currently embedded in inconsistent file names or demonstration recordings.

Reference checks used to identify open items

These checks were used only to flag points requiring faculty adjudication; they do not replace CUHK's approved references:

Methotrexate tablets prescribing information: <https://dailymed.nlm.nih.gov/dailymed/drugInfo.cfm?setid=948e0bab-6524-4b87-9853-1799d4415558>

CRESTOR (rosuvastatin) prescribing information:

<https://www.dailymed.nlm.nih.gov/dailymed/drugInfo.cfm?setid=3bf80420-7482-44c0-a117-30793ba89544>

Hong Kong Drug Office consumer FAQ:

https://www.drugoffice.gov.hk/eps/do/en/consumer/news_informations/faq.html

Document status

Updated working draft for faculty review, incorporating the latest CUHK calibration comments for the specified Diuretics and Methotrexate video segments. No backend API, numeric grade model or remaining open scoring rule is represented as approved by CUHK at this stage.